

ART026001P
SORAFENIB MY 200MG 6X10S FCT PIL



GENERAL INFORMATION

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REGULATORY APPROVAL

RENOPLAST
200 mg FILM-COATED TABLETS
SORAFENIB

1. NAME OF THE MEDICINAL PRODUCT

RENOPLAST 200 mg FILM-COATED TABLETS (Sorafenib)

2. QUALITATIVE AND QUANTITATIVE COMPOSITION

Each film-coated tablet contains 200 mg of sorafenib (as tosylate). For the full list of excipients, see section 6.1.

3. PHARMACEUTICAL FORM

Film-coated tablet (tablet).
Red-brown, round, biconvex film-coated tablets, debossed with "200" on one side and plain on the other side.

4. CLINICAL PARTICULARS

4.1 Therapeutic indications

RENOPLAST is indicated for the treatment of patients with hepatocellular carcinoma.

RENOPLAST is indicated for the treatment of patients with advanced renal cell carcinoma who have failed prior interferon- α or interleukin-2 based therapy or are considered unsuitable for such therapy.

RENOPLAST is indicated for the treatment of patients with locally advanced or metastatic, progressive, differentiated thyroid carcinoma refractory to radioactive iodine.

4.2 Posology and method of administration

Recommended dose

The recommended daily dose of sorafenib is 400 mg (2 x 200 mg tablets) taken twice a day, either without food or together with a low fat or moderate fat meal.

Method of administration

For oral use. To be swallowed with a glass of water.

Dosage regimen

Duration of treatment

Treatment should be continued until the patient is no longer clinically benefiting from therapy or until unacceptable toxicity occurs.

Dose titration, dose adjustment, special monitoring advice

Dose Reduction for Hepatocellular Carcinoma and advanced Renal Cell Carcinoma.

Management of suspected adverse drug reactions may require temporary interruption and/or dose reduction of sorafenib therapy. When dose reduction is necessary during the treatment of hepatocellular carcinoma (HCC) and advanced renal cell carcinoma (RCC), the sorafenib dose should be reduced to two tablets of 200 mg once daily (see section Special warnings and precautions for use).
Suggested dose modifications for dermatologic toxicities are outlined in Table 1.

Table 1: Suggested Dose Modifications for Dermatologic Toxicities in Patients with Hepatocellular or Renal Cell Carcinoma

Dermatologic Toxicity Grade	Occurrence	Suggested Dose Modification
Grade 1: Numbness, dysesthesia, paresthesia, tingling, painless swelling, erythema or discomfort of the hands or feet which does not disrupt the patient's normal activities	Any occurrence	Continue treatment with RENOPLAST and consider topical therapy for symptomatic relief
Grade 2: Painful erythema and swelling of the hands or feet and/or discomfort affecting the patient's normal activities	1 occurrence	Continue treatment with RENOPLAST and consider topical therapy for symptomatic relief. If no improvement within 7 days, see below
	No improvement within 7 days or 2 nd or 3 rd occurrence	Interrupt RENOPLAST treatment until toxicity resolves to Grade 0-1 When resuming treatment, decrease RENOPLAST dose by one dose level (400 mg daily or 400 mg every other day)
	4 th occurrence	Discontinue RENOPLAST treatment
Grade 3: Moist desquamation, ulceration, blistering or severe pain of the hands or feet, or severe discomfort that causes the patient to be unable to work or perform activities of daily living	1 st or 2 nd occurrence	Interrupt RENOPLAST treatment until toxicity resolves to Grade 0-1 When resuming treatment, decrease RENOPLAST dose by one dose level (400 mg daily or 400 mg every other day)
	3 rd occurrence	Discontinue RENOPLAST treatment

Dose Reduction for Differentiated Thyroid Carcinoma

Management of suspected adverse drug reactions may require temporary interruption and/or dose reduction of sorafenib therapy.

When dose reduction is necessary during the treatment of differentiated thyroid carcinoma, the sorafenib dose should be reduced to 600mg daily in divided doses (two tablets of 200mg and one tablet of 200mg twelve hours apart).

If additional dose reduction is necessary, sorafenib may be reduced to one tablet of 200mg twice daily, followed by one tablet of 200mg once daily. After improvement of non-hematological adverse reactions, the dose of sorafenib may be increased.

Table 2: Recommended Dose Modifications for Dermatologic Toxicities for Patients with Differentiated Thyroid Carcinoma:

Dermatologic Toxicity Grade	Occurrence	RENOPLAST Dose Modification
Grade 1: Numbness, dysesthesia, paresthesia, tingling, painless swelling, erythema or discomfort of the hands or feet which does not disrupt the patient's normal activities	Any occurrence	Continue treatment with RENOPLAST
Grade 2: Painful erythema and swelling of the hands or feet and/or discomfort affecting the patient's normal activities	1 st occurrence	Decrease RENOPLAST dose to 600 mg daily if no improvement within 7 days, see below.
	No improvement within 7 days at reduced dose or 2 nd occurrence	Interrupt RENOPLAST until resolved or improved to grade 1 If RENOPLAST is resumed, decrease dose. (See "Dose Reduction for Differentiated Thyroid Carcinoma").
	3 rd occurrence	Interrupt RENOPLAST until resolved or improved to grade 1 If RENOPLAST is resumed, decrease dose. (See "Dose Reduction for Differentiated Thyroid Carcinoma").
	4 th occurrence	Discontinue RENOPLAST permanently.
Grade 3: Moist desquamation, ulceration, blistering, or severe pain of the hands or feet, resulting in inability to work or perform activities of daily living	1 st occurrence	Interrupt RENOPLAST until resolved or improved to grade 1 If RENOPLAST is resumed, decrease dose by one dose level. (See "Dose Reduction for Differentiated Thyroid Carcinoma").
	2 nd occurrence	Interrupt RENOPLAST until resolved or improved to grade 1 When RENOPLAST is resumed, decrease dose by 2 dose levels. (See "Dose Reduction for Differentiated Thyroid Carcinoma").
	3 rd occurrence	Discontinue RENOPLAST permanently

Additional information on special populations

Pediatric Patients

The safety and effectiveness of sorafenib in pediatric patients has not been established.

Geriatric patients

Elderly (above 65 years), Gender and Body Weight

No dose adjustment is required on the basis of patient age (above 65 years), gender, or body weight.

Patients with hepatic impairment

No dose adjustment is required in patients with Child-Pugh A or B hepatic impairment: Sorafenib has not been studied in patients with Child-Pugh C hepatic impairment (see section Pharmacokinetic Properties – Pharmacokinetics in special populations – Hepatic Impairment).

Patients with renal impairment

No dose adjustment is required in patients with mild, moderate or severe renal impairment not requiring dialysis. Sorafenib has not been studied in patients undergoing dialysis (see section Pharmacokinetic Properties –Special populations – Renal impairment).
Monitoring of fluid balance and electrolytes in patients at risk of renal dysfunction is advised.

4.3 Contraindications

Hypersensitivity to the active substance or to any of the excipients listed in section 6.1.

Sorafenib in combination with carboplatin and paclitaxel is contraindicated in patients with squamous cell lung cancer [see section 4.3]

4.4 Special warnings and precautions for use

Dermatological toxicities

Hand foot skin reaction (palmar-plantar erythrodysesthesia) and rash represent the most common adverse drug reactions with sorafenib. Rash and hand foot skin reaction are usually CTC (Common Toxicity Criteria) Grade 1 and 2 and generally appear during the first six weeks of treatment with sorafenib. Management of dermatological toxicities may include topical therapies for symptomatic relief, temporary treatment interruption and/or dose modification of sorafenib, or in severe or persistent cases, permanent discontinuation of sorafenib (see section 4.8).

Hypertension

An increased incidence of arterial hypertension was observed in sorafenib-treated patients. Hypertension was usually mild to moderate, occurred early in the course of treatment, and was amenable to management with standard antihypertensive practice. Blood pressure should be monitored regularly and treated, if required, in accordance with standard medical practice. In cases of severe or persistent hypertension, or hypertensive crisis despite institution of antihypertensive therapy, permanent discontinuation of sorafenib should be considered (see section 4.8).

Aneurysms and artery dissections

The use of VEGF pathway inhibitors in patients with or without hypertension may promote the formation of aneurysms and/or artery dissections. Before initiating RENOPLAST, this risk should be carefully considered in patients with risk factors such as hypertension or history of aneurysm.

Hypoglycaemia

Decreases in blood glucose, in some cases clinically symptomatic and requiring hospitalization due to loss of consciousness, have been reported during sorafenib treatment. In case of symptomatic hypoglycaemia, sorafenib should be temporarily interrupted. Blood glucose levels in diabetic patients should be checked regularly in order to assess if anti-diabetic medicinal product's dosage needs to be adjusted.

Haemorrhage

An increased risk of bleeding may occur following sorafenib administration. If any bleeding event necessitates medical intervention, it is recommended that permanent discontinuation of sorafenib should be considered (see section 4.8).

Cardiac ischaemia and/or infarction

Temporary or permanent discontinuation of sorafenib should be considered in patients who develop cardiac ischaemia and/or infarction (see section 4.8).

QT interval prolongation

Sorafenib has been shown to prolong the QT/QTc interval (see section 5.1), which may lead to an increased risk for ventricular arrhythmias. Use sorafenib with caution in patients who have, or may develop prolongation of QTc, such as patients with a congenital long QT syndrome, patients treated with a high cumulative dose of anthracycline therapy, patients taking certain anti-arrhythmic medicines or other medicinal products that lead to QT prolongation, and those with electrolyte disturbances such as hypokalaemia, hypocalcaemia, or hypomagnesaemia. When using sorafenib in these patients, periodic monitoring with on-treatment electrocardiograms and electrolytes (magnesium, potassium, calcium) should be considered.

Gastrointestinal perforation

Gastrointestinal perforation is an uncommon event and has been reported in patients taking sorafenib. In some cases this was not associated with apparent intra-abdominal tumour. Sorafenib therapy should be discontinued (see section 4.8).

Tumor Lysis Syndrome

Cases of TLS, some fatal, have been reported in postmarketing surveillance in patients treated with sorafenib. Risk factors for TLS include high tumour burden, pre-existing chronic renal insufficiency, oliguria, dehydration, hypotension, and acidic urine. These patients should be monitored closely and treated promptly as clinically indicated, and prophylactic hydration should be considered.

Hepatic impairment

No data is available on patients with Child Pugh C (severe) hepatic impairment. Since sorafenib is mainly eliminated via the hepatic route exposure might be increased in patients with severe hepatic impairment (see sections 4.2 and 5.2).

Warfarin co-administration

Infrequent bleeding events or elevations in the International Normalised Ratio (INR) have been reported in some patients taking warfarin while on sorafenib therapy. Patients taking concomitant warfarin or phenprocoumon should be monitored regularly for changes in prothrombin time, INR or clinical bleeding episodes (see sections 4.5 and 4.8).

Wound healing complications

No data of the effect of sorafenib on wound healing is available. Temporary interruption of sorafenib therapy is recommended for precautionary reasons in patients undergoing major surgical procedures. There is limited clinical experience regarding the timing of reinitiation of therapy following major surgical intervention. Therefore, the decision to resume sorafenib therapy following a major surgical intervention should be based on clinical judgement of adequate wound healing.

Elderly population

Cases of renal failure have been reported. Monitoring of renal function should be considered.

Drug-drug interactions

Caution is recommended when administering sorafenib with compounds that are metabolised/eliminated predominantly by the UGT1A1 (e.g. irinotecan) or UGT1A9 pathways (see section 4.5).

Caution is recommended when sorafenib is co-administered with docetaxel (see section 4.5).

Co-administration of neomycin or other antibiotics that cause major ecological disturbances of the gastrointestinal microflora may lead to a decrease in sorafenib bioavailability (see section 4.5). The risk of reduced plasma concentrations of sorafenib should be considered before starting a treatment course with antibiotics.

Higher mortality has been reported in patients with squamous cell carcinoma of the lung treated with sorafenib in combination with platinum-based chemotherapies. No single cause of death dominated, but higher incidence of respiratory failure, hemorrhages and infectious adverse events were observed in patients treated with sorafenib as add-on to platinum-based chemotherapies.

Disease specific warnings

Differentiated thyroid cancer (DTC)

Before initiating treatment, physicians are recommended to carefully evaluate the prognosis in the individual patient considering maximum lesion size (see section 5.1), symptoms related to the disease (see section 5.1) and progression rate. Management of suspected adverse drug reactions may require temporary interruption or dose reduction of sorafenib therapy. Dose reductions were only partially successful in alleviating adverse reactions. Therefore, repeat evaluations of benefit and risk is recommended taking anti-tumour activity and tolerability into account.

Haemorrhage in DTC

Due to the potential risk of bleeding, tracheal, bronchial, and oesophageal infiltration should be treated with localized therapy prior to administering sorafenib in patients with DTC.

Hypocalcaemia in DTC

When using sorafenib in patients with DTC, close monitoring of blood calcium level is recommended. Hypocalcaemia was more frequent and more severe in patients with DTC, especially with a history of hypoparathyroidism, compared to patients with renal cell or hepatocellular carcinoma. Hypocalcaemia grade 3 and 4 occurred in sorafenib-treated patients with DTC (see section 4.8). Severe hypocalcaemia should be corrected to prevent complications such as QT-prolongation or torsade de pointes (see section QT prolongation).

TSH suppression in DTC

Increases in TSH levels above 0.5mU/L were observed in sorafenib treated patients. When using sorafenib in DTC patients, close monitoring of TSH level is recommended.

Renal cell carcinoma

Benefit/risk in these patients has not been evaluated.

4.5 Interaction with other medicinal products and other forms of interaction

Inducers of metabolic enzymes

Administration of rifampicin for 5 days before administration of a single dose of sorafenib resulted in reduction of sorafenib AUC. Other inducers of CYP3A4 activity and/or glucuronidation (e.g. Hypericum perforatum also known as St. John's wort, phenytoin, carbamazepine, phenobarbital, and dexamethasone) may also increase metabolism of sorafenib and thus decrease sorafenib concentrations.

CYP3A4 inhibitors

Ketoconazole, a potent inhibitor of CYP3A4, administered once daily for 7 days did not alter the mean AUC of a single 50 mg dose of sorafenib. These data suggest that clinical pharmacokinetic interactions of sorafenib with CYP3A4 inhibitors are unlikely.

CYP2B6, CYP2C8 and CYP2C9 substrates

Sorafenib inhibited CYP2B6, CYP2C8 and CYP2C9 with similar potency. However, concomitant administration of sorafenib 400 mg twice daily with cyclophosphamide, a CYP2B6 substrate, or paclitaxel, a CYP2C8 substrate, did not result in a clinically meaningful inhibition. These data suggest that clinical pharmacokinetic interactions of sorafenib with CYP3A4 inhibitors are unlikely.

Additionally, concomitant treatment with sorafenib and warfarin, a CYP2C9 substrate, did not result in changes in mean PT-INR compared to placebo. Thus, also the risk for a clinically relevant inhibition of CYP2C9 by sorafenib may be expected to be low. However, patients taking warfarin or phenprocoumon should have their INR checked regularly (see section 4.4).

CYP3A4, CYP2D6 and CYP2C19 substrates

Concomitant administration of sorafenib and midazolam, dextromethorphan or omeprazole, which are substrates for cytochromes CYP3A4, CYP2D6 and CYP2C19 respectively, did not alter the exposure of these agents. This indicates that sorafenib is neither an inhibitor nor an inducer of these cytochrome P450 isoenzymes. Therefore, clinical pharmacokinetic interactions of sorafenib with substrates of these enzymes are unlikely.

UGT1A1 and UGT1A9 substrates

Sorafenib inhibited glucuronidation via UGT1A1 and UGT1A9. The clinical relevance of this finding is unknown (see below and section 4.4).

Data of CYP enzyme induction

CYP1A2 and CYP3A4 activities were not altered after treatment of cultured human hepatocytes with sorafenib, indicating that sorafenib is unlikely to be an inducer of CYP1A2 and CYP3A4.

P-gp-substrates

Sorafenib has been shown to inhibit the transport protein p-glycoprotein (P-gp). Increased plasma concentrations of P-gp substrates such as dipiox cannot be excluded with concomitant treatment with sorafenib.

Combination with other anti-neoplastic agents

Sorafenib had no clinically relevant effect on the pharmacokinetics of gemitabine, cisplatin, carboplatin, oxaliplatin or cyclophosphamide.

Paclitaxel/carboplatin

O Administration of paclitaxel (225 mg/m²) and carboplatin with sorafenib ($\leq$ 400 mg twice daily), administered with a 3-day break in sorafenib dosing (two days prior to and on the day of paclitaxel/carboplatin administration), resulted in no significant effect on the pharmacokinetics of paclitaxel.

O Co-administration of paclitaxel (225 mg/m², once every 3 weeks) and carboplatin with sorafenib (400 mg twice daily, without a break in sorafenib dosing) resulted in an increase in sorafenib exposure, an increase in paclitaxel exposure and an increase in 6-OH paclitaxel exposure. The pharmacokinetics of carboplatin were unaffected.

These data indicate no need for dose adjustments when paclitaxel and carboplatin are co-administered with sorafenib with a 3-day break in sorafenib dosing (two days prior to and on the day of paclitaxel/carboplatin administration). The clinical significance of the increases in sorafenib and paclitaxel exposure, upon co-administration of sorafenib without a break in dosing, is unknown.

Capecitabine

Co-administration of capecitabine (750-1050 mg/m² twice daily, Days 1-14 every 21 days) and sorafenib (200 or 400 mg twice daily, continuous uninterrupted administration) resulted in no significant change in sorafenib exposure, but an increase in capecitabine exposure and an increase in 5-FU exposure. The clinical significance of these small to modest increases in capecitabine and 5-FU exposure when co-administered with sorafenib is unknown.

Doxorubicin/irinotecan

Concomitant treatment with sorafenib resulted in an increase in the AUC of doxorubicin. When administered with irinotecan, whose active metabolite SN-38 is further metabolised by the UGT1A1 pathway, there was an increase in the AUC of SN-38 and an increase in the AUC of irinotecan. The clinical significance of these findings is unknown (see section 4.4).

Docetaxel

Docetaxel (75 or 100 mg/m² administered once every 21 days) when co-administered with sorafenib (200 mg twice daily or 400 mg twice daily administered on Days 2 through 19 of a 21-day cycle with a 3-day break in dosing around administration of docetaxel) resulted in an increase in docetaxel AUC and an increase in docetaxel C_{max}. Caution is recommended when sorafenib is co-administered with docetaxel (see section 4.4).

Combination with other agents

Neomycin

Co-administration of neomycin, a non-systemic antimicrobial agent used to eradicate gastrointestinal flora, interferes with the enterohepatic recycling of sorafenib (see section 5.2, Metabolism and Elimination), resulting in decreased sorafenib exposure. There is no data of Effects of other antibiotics, but will likely depend on their ability to interfere with microorganisms with glucuronidase activity.

4.6 Fertility, pregnancy and lactation

Pregnancy

There are no data on the use of sorafenib in pregnant women. Data in animals have shown reproductive toxicity including malformations. In rats, sorafenib and its metabolites were demonstrated to cross the placenta and sorafenib is anticipated to cause harmful effects on the foetus. Sorafenib should not be used during pregnancy unless clearly necessary, after careful consideration of the needs of the mother and the risk to the foetus. Women of childbearing potential must use effective contraception during treatment.

Lactation

It is not known whether sorafenib is excreted in human milk. In animals, sorafenib and/or its metabolites were excreted in milk. Because sorafenib could harm infant growth and development, women must not breast-feed during sorafenib treatment.

Fertility

Results from animal data further indicate that sorafenib can impair male and female fertility.

4.7 Effects on ability to drive and use machines

No data on the effects on the ability to drive and use machines is available. There is no evidence that sorafenib affects the ability to drive or to operate machinery.

4.8 Undesirable effects

The most important serious adverse reactions were myocardial infarction/schaemia, gastrointestinal perforation, drug induced hepatitis, haemorrhage, and hypertension/hypertensive crisis. The most common adverse reactions were diarrhoea, fatigue, alopecia, infection, hand foot skin reaction (corresponds to palmar plantar erythrodysesthesia syndrome in MedDRA) and rash. Adverse reactions reported are listed below in table 3, by system organ class (in MedDRA) and frequency. Frequencies are defined as: very common, common, uncommon, rare, not known. Within each frequency grouping, undesirable effects are presented in order of decreasing seriousness.

Table 3: All adverse reactions reported in patients

System organ class	Very common	Common	Uncommon	Rare	Not known
Infections and infestations	infection	folliculitis			
Blood and lymphatic system disorders	lymphopenia	leucopenia neutropenia anaemia thrombocytopenia			
Immune system disorders			hypersensitivity reactions (including skin reactions and urticaria), anaphylactic reaction	angioedema	
Endocrine disorders		hypothyroidism	hyperthyroidism		
Metabolism and nutrition disorders	Anorexia, hypo-phosphataemia	hypocalcaemia hypokalaemia hyponatraemia hypoglycaemia	dehydration		
Psychiatric disorders		depression			
Nervous system disorders		peripheral sensory neuropathy dysgeusia	reversible posterior leukoencephalopathy*		encephalopathy ^o
Ear and labyrinth disorders		tinnitus			
Cardiac disorders		congestive heart failure* myocardial ischaemia and infarction*		QT prolongation	
Vascular disorders	haemorrhage (inc. gastrointestinal*, respiratory tract* and cerebral haemorrhage*), hypertension	flushing	hypertensive crisis*		aneurysms and artery dissections
Respiratory, thoracic and mediastinal disorders		rhinorrhoea dysphonia	interstitial lung disease-like events* (pneumonitis, radiation pneumonitis, acute respiratory distress, etc.)		
Gastrointestinal disorders	Diarrhoea, nausea, vomiting, constipation	stomatitis (including dry mouth and glossodynia) dyspepsia dysphagia gastro oesophageal reflux disease	pancreatitis gastritis gastrointestinal perforations*		
Hepatobiliary disorders			increase in bilirubin and jaundice, cholecystitis, cholangitis	drug induced hepatitis*	
Skin and subcutaneous tissue disorders	dry skin rash, alopecia hand foot skin reaction** erythema pruritus	keratoacanthoma/squamous cell cancer of the skin dermatitis exfoliative acne skin desquamation hyperkeratosis	eczema erythema multiforme	radiation recall dermatitis, Stevens-Johnson syndrome, leucocytoclastic vasculitis, toxic epidermal necrolysis*	
Musculo-skeletal and connective tissue disorders	arthralgia	myalgia muscle spasms		rhabdomyolysis	
Renal and urinary disorders		renal failure proteinuria		nephrotic syndrome	
Reproductive system and breast disorders		erectile dysfunction	gynaecomastia		
General disorders and administration site conditions	fatigue pain (including mouth, abdominal, bone, tumour pain and headache), fever	asthenia influenza like illness mucosal inflammation			
Investigations	weight decreased, increased amylase, increased lipase	transient increase in transaminases	transient increase in blood alkaline phosphatase INR abnormal, prothrombin level abnormal		

* The adverse reactions may have a life-threatening or fatal outcome. Such events are either uncommon or less frequent than uncommon.
** Hand foot skin reaction corresponds to palmar plantar erythrodysesthesia syndrome in MedDRA.
^o Cases have been reported in the post marketing setting.

Further information on selected adverse drug reactions

Congestive heart failure

Congestive heart failure was reported as an adverse event in patients treated with sorafenib

Additional information on special populations

Certain adverse drug reactions such as hand foot skin reaction, diarrhoea, alopecia, weight decrease, hypertension, hypocalcaemia, and keratoacanthoma/squamous cell carcinoma of skin occurred at a substantially higher frequency in patients with differentiated thyroid compared to patients in the renal cell or hepatocellular carcinoma studies.

